DIAZ ALVARADO, Francisca **DOB:** 10/03/1969 (54 yo F) **Acc No.** 17107 **DOS:** 03/05/2024

DIAZ ALVARADO, Francisca

54 Y old Female, DOB: 10/03/1969
Account Number: 17107
1036 BROWN DUCK TRL, LAWRENCEVILLE, GA-30043-5205
Home: 951-396-6295
Guarantor: Diaz Alvarado, Francisca Insurance: Medical Lien
Payer ID: PAPER
Appointment Facility: THRIVE MP - LILBURN

03/05/2024 **PROGRESS NOTE: SHIEL PATEL, MD**

Current Medications

Taking

- Ibuprofen 200 MG Tablet 1 tablet with food or milk as needed Orally Three times a day

Medication List reviewed and reconciled with the patient

Past Medical History

Asthma.

Surgical History

Denies Past Surgical History

Family History

Father: deceased
Mother: deceased
no known history.

Social History

Tobacco Use:
Tobacco Use/Smoking Are you a current smoker.
Drugs/Alcohol:
Do you drink alcohol?: Socially.
Miscellaneous:
Marital status: single. Occupation: works in AC.

Allergies

N.K.D.A.

Hospitalization/Major Diagnostic Procedure

Denies Past Hospitalization

Review of Systems

General/Constitutional:
Patient denies fatigue , headache , sleep disturbance.
Allergy/Immunology:
Patient denies rash , itching.
Ophthalmologic:
Patient denies blurred vision ,

Reason for Appointment

1. NP: Neck and Low back pain

History of Present Illness

Consultation:
Chief Complaint: Francisca Diaz Alvarado is a 54-year-old female. Right-hand dominance. She is currently employed in AC technician. She continues to work despite sustained injuries. She was seen today as a new patient for injuries sustained to her neck and low back as a result of a motor vehicle accident that occurred on 2/20/2024. She was a restrained driver involved in a rear-ended collision. The airbags did not deploy. She denies loss of consciousness or head injury at the time of the collision. She has attended 2 sessions of chiropractic care with minimal relief. She is currently taking Ibuprofen with moderate relief. She denies sleep issues. The patient states that the pain is affecting her ability to perform regular day-to-day activities. The patient denies previous pain in the mentioned areas. Medical Assistant: Ashley Valladares, CCMA . Translator present: Ashley Valladares, CCMA .
Lumbar Spine/Lower Back:
Low back pain The patient states that her low back pain is her primary pain complaint today. She rates the pain a 7 on a scale from 1 to 10. She states that the pain in her low back is intermittent and describes the pain as aching. She states the pain worsens with prolonged sitting and is improved by medications. She admits referred pain from the low back into the bilateral hips and lower extremities with numbness and tingling in the toes. The patient denies bowel and bladder changes. The patient notes her low back pain has remained the same since the accident.
Cervical Spine/Neck:
Neck pain The patient states that her neck pain is her second pain complaint today. She rates the pain a 6 on a scale from 1 to 10. She states that the pain in her neck is intermittent and describes the pain as aching and throbbing. She states the neck pain is localized at the base of the neck. She states the pain worsens with axial rotation and extension and is improved by taking medications. She reports referred pain from her neck to the bilateral shoulders and upper extremities with numbness and tingling. She denies any headaches, nausea, vomiting, blurred vision, dizziness, loss of balance, or ringing in the ears. The patient notes her neck pain has remained the same since the accident.

Vital Signs

Note generated by eClinicalWorks EMR/PM Software (www.eClinicalWorks.com)

DIAZ ALVARADO, Francisca **DOB:** 10/03/1969 (54 yo F) **Acc No.** 17107 **DOS:** 03/05/2024

double vision , eye pain.
ENT:
Patient denies decreased hearing , pain.
Endocrine:
Patient denies excessive thirst , frequent urination.
Respiratory:
Patient denies chest pain , shortness of breath, wheezing .
Cardiovascular:
Patient denies heart problems .
Gastrointestinal:
Patient denies Incontinence of bowels, acid reflux , vomiting , blood in stool , change in bowel habits.
Hematology:
Patient denies bleeding disorders , blood clot .
Genitourinary:
Patient denies incontinence of urine , blood in the urine.
Musculoskeletal:
Patient denies leg cramps , joint pain, knee pain. Back Pain Admits. Hip Pain Admits. Neck Pain Admits. Shoulder Pain Admits.
Peripheral Vascular:
Patient denies decreased sensation in extremities.
Podiatric:
Patient denies burning , difficulty walking , foot numbness.
Skin:
Patient denies rash , wounds , incisions.
Neurologic:
Patient denies balance difficulty, loss of strength, insomnia , memory loss. Tingling/Numbness admits.
Psychiatric:
Patient denies anxiety , depressed mood , stressors, suicidal thoughts .

BP: **141/84** mm Hg, HR: **73** /min, RR: **17** /min, Oxygen sat %: **97** %, Temp: **98.3** F, Ht: 59 in, Wt: **122.7** lbs, BMI: **24.78** Index, Ht-cm: 149.86, Wt-kg: 55.66 kg.

Examination

Physical Examination:
EXAM:
General Appearance: Well nourished, well developed, and in no acute distress. Alert and oriented x3
EYES: Conjunctiva normal, EOM intact
EARS: No deformities, hearing grossly intact
CHEST: No chest wall deformities
SKIN: No abrasions or contusions
.
Cervical Spine/Neck:
C SPINE EXAM:
CERVICAL SPINE:
I see no obvious scars, deformities, redness, or edema. There is tenderness to palpation at the bilateral cervical paraspinal muscles and bilateral trapezius muscles.
ROM: 45degree of left lateral bending, 45degree of right lateral bending, 60degree of left cervical rotation, 60degree of right cervical rotation, 60degree of forward cervical flexion, 60degree of cervical extension. There is pain with cervical flexion and lateral rotation bilaterally.
Strength Right Upper Extremity: 5/5 shoulder abduction strength, 5/5 wrist extension strength, 5/5 wrist flexion strength, 5/5 finger flexion strength, 5/5 finger abduction/adduction strength.
Strength Left Upper Extremity: 5/5 shoulder abduction strength, 5/5 wrist extension strength, 5/5 wrist flexion strength, 5/5 finger flexion strength, 5/5 finger abduction/adduction strength.
Reflexes: 2+ biceps tendon, 2+ brachioradialis tendon, and 2+ triceps tendon.
Sensation: Normal sensation in all fingers. Normal sensation in the dermatomes tested.
Vascular: Brisk capillary refill in all fingers
.
Lumbar Spine/Lower back:
Lumbar Spine Examination:
LUMBAR SPINE:
The patient describes maximal pain at lower lumbar area. I see no scoliosis in the lumbar and thoracic spine.
Palpation: There is tenderness to palpation at the bilateral lumbar paraspinal muscles. Both SI joints were tender to palpation. There is tenderness to palpation over the lumbar facet joints.
ROM: 25degree of left lateral bending, 25degree of right lateral bending, 60degree of forward lumbar flexion, 25degree of lumbar extension. Patient had most discomfort with lumbar flexion/extension with external rotation (facet loading).
Special Tests: There is positive straight leg raise bilaterally. Bilateral FABER test is positive, bilateral thigh thrust test is positive, bilateral pelvic compression testing is positive
Strength Right Lower Extremity: 5/5 hip flexion, 5/5 knee extension, 5/5 ankle dorsiflexion, 5/5 great toe extension, 5/5 ankle plantar flexion.

Strength Left Lower Extremity: 5/5 hip flexion, 5/5 knee extension, 5/5 ankle dorsiflexion, 5/5 great toe extension, 5/5 ankle plantar flexion.
Reflexes: 2+ patellar tendon reflex, 2+ Achilles tendon reflex bilaterally.
Sensation: Normal sensory bilaterally in the dermatomes tested.
Vascular: Both calves are supple with a negative Homans sign bilaterally. There are no clinical signs for DVT
.

Assessments

1. Sprain of ligaments of cervical spine, initial encounter - S13.4XXA (Primary)
2. Sprain of ligaments of lumbar spine, initial encounter - S33.5XXA
3. Cervical paraspinal muscle spasm - M62.838
4. Lumbar facet joint pain - M54.59
5. Lumbar paraspinal muscle spasm - M62.830
6. Pain of cervical facet joint - 723.1
7. Sacroiliitis - 720.2
8. Radiculopathy, cervical region - M54.12
9. Radiculopathy, lumbar region - M54.16

Treatment

1. Sprain of ligaments of cervical spine, initial encounter
Referral To:Radiology
Reason:MRI cervical and lumbar spine without contrast

Referral To:Chiropractor
Reason:Continue chiropractic care

2. Sprain of ligaments of lumbar spine, initial encounter
Referral To:Radiology
Reason:MRI cervical and lumbar spine without contrast

Referral To:Chiropractor
Reason:Continue chiropractic care

3. Others
Start Meloxicam Tablet, 15 MG, 1 tablet, Orally, Once a day, 30 day(s), 30
Start Metaxalone Tablet, 800 MG, 1 tablet, Orally, Three times a day, 30 day(s), 90
Notes: The patient presents today as a new patient involved in a motor vehicle accident that occurred on 02/20/2024 sustaining injuries to their cervical and lumbar spine. The patient has also attended 2 sessions of chiropractic care noting minimal improvement in their overall pain symptoms. The patient reports no pain in the cervical or lumbar spine prior to the motor vehicle accident. Today we will evaluate and develop a treatment plan.

The patient's primary pain complaint is the lumbar spine.
-The patient reports radicular symptoms of numbness and tingling down both lower extremities.
-Upon physical examination today, the patient had tenderness to palpation over the lumbar paraspinal musculature, lumbar facet joints, sacroiliac joints, and midline region of the lumbar spine. When performing special testing of the lumbar spine there were multiple clinical findings for

reproduction of pain symptoms to the sacroiliac joints. The patient also had reproduction of pain symptoms to the lumbar facet joints. When performing lower motor neurological testing no neurological deficits were found. However, the patient did have a positive straight leg raise test bilaterally causing reproduction of radicular symptoms down both lower extremities.
-Based on exam findings I suspect that the majority of patient's lumbar spine pain is likely due to facetogenic injury, Inflammation of the sacroiliac joints, spraining the ligaments of the lumbar spine, and potentially lumbar disc pathology. I recommend we order an MRI of the lumbar spine to further evaluate.
-I recommend the patient continue with chiropractic therapy.
-I recommend the patient try an anti-inflammatory and a muscle relaxer to help alleviate some of their pain symptoms. I will dispense meloxicam and metaxalone today.
-I recommend the patient wear a lumbar corset brace to further support the lumbar spine. I will dispense a lumbar corset brace to the patient today.

The patient's secondary pain complaint is the cervical spine.
-The patient reports radicular symptoms of numbness and tingling down both upper extremities.
-Upon physical examination today, The patient had tenderness to palpation over the cervical paraspinal musculature, trapezius musculature, and midline region of the cervical spine. The patient did have decreased range of motion of the cervical spine. The patient did have reproduction of pain symptoms with cervical facet loading. When performing upper motor neurological testing no neurological deficits were found.
-Based on exam findings I suspect that the majority of the patient's cervical spine pain is likely combination of myofascial muscle sprain/strain and discogenic injury. However, given that there is also pain with cervical facet loading, I cannot rule out the possibility that cervical facet joints are also contributing. I recommend we order an MRI of the cervical spine to further evaluate.
-I recommend the patient continue with chiropractic therapy.

Given that the patient denies any previous history of cervical or lumbar spine pain prior to the motor vehicle accident, it is within reasonable degree of medical certainty that the patient's symptoms or exam findings are likely due to motor vehicle accident on 02/20/2024.

Work Status: The patient currently works full-time and was given a work status today of SEDENTARY/LIGHT work restrictions. Lifting 10lbs max and occasionally lifting/carrying of paperwork or small tools. These restrictions are to remain in effect until the next follow-up appointment.

The patient will return in 2 weeks to reevaluate and review their MRI findings. The patient verbalized understanding of the current treatment plan and was amenable. All questions were answered. If the patient has any additional questions, the patient can call the office for details.

Durable medical equipment dispensed:
LSO BRACE
Utilizing accepted medical practice standards, a lumbar brace was given to this patient. The brace is essential in the treatment of the patients injury

and will aid the patient in returning to pre-injury activities of daily living. The expected benefits from using the lumbar brace are: reduction in pain by restricting mobility of the lumbar area, facilitate healing to the affected muscles and the surrounding soft tissues; support weak spinal muscles while weightbearing.

Medications dispensed:
The patient was dispensed metaxalone 800 mg, 60 tablets. I want the patient to take 1 tablet before bedtime.
The patient was dispensed meloxicam 15 mg, 30 tablets. I want the patient to take 1 tablet daily.
Medication disclaimer:
I want the patient to take an oral NSAID. The patient denies history of gastroesophageal reflux disease or bleeding ulcers. I told the patient to take the NSAID after a large meal to avoid any GI symptoms. I want the patient to take an oral muscle relaxer. I instructed the patient to take this medication before bedtime because it may cause somnolence. The patient was instructed about the medication dosage and was told not to take more medications than prescribed. Risks and benefits of each medication was discussed with the patient. The patient was instructed to discontinue any medication immediately if the patient experiences any negative side effects or allergic reactions.

VOICE DICTATION DISCLAIMER
This document was created using voice recognition software. Any mistakes in the transcription are unintentional. An attempt was made to proofread for accuracy, but to expedite availability, some errors may persist. Please contact our Medical Records Dept. with any need for correction or further clarification.

Procedure Codes

L0627 LO SAGITT RIGID PANEL PREFAB
99070 Meloxicam 15mg Tab #30
99070 Metaxalone 800mg #60

Follow Up

2 Weeks (Reason: MRI review (cervical + lumbar spine))

Electronically signed by SHIEL PATEL , M.D. on 03/05/2024 at 07:27 PM EST

Sign off status: Completed

THRIVE MP - LILBURN
3970 FIVE FORKS TRICKUM RD SW
Suite A
LILBURN, GA 30047-2339
Tel: 678-257-2547
Fax: 866-317-9099

American Health Imaging of Decatur
2774 North Decatur Rd
Decatur, GA 30033
Phone #: (404) 292-2277
Fax: (404) 292-2294

Name: Franciscia Alvarado
DOB: 10/3/1969
Gender: Female
Acc #: 31853265
Referrer: Pulido, Carlos, MD
Exam Name: MR Cervical Spine wo IV Contrast | 72141
Exam Date: 3/7/2024 03:21 PM
Secondary ID: MPI5555266
SID: A31853265

EXAM: Cervical Spine MRI without contrast.

HISTORY: 54-year-old female with neck pain that radiates to the shoulders, with associated tingling in the arms. MVA mid to late February 2024.

COMPARISON: None.

TECHNIQUE: Multiplanar, multisequence standard MRI evaluation of the cervical spine without contrast.

FINDINGS: There is some motion degradation.

There is mild flattening of mid cervical lordosis. Mild apex left scoliosis. No fracture or destructive process. No intrinsic cord lesion. Cervicomedullary junction is anatomic. Visualized soft tissues of the neck are unremarkable. Cervical segment vertebral arteries are patent, mildly tortuous.

The odontoid process is intact. Moderate degenerative narrowing of the median atlantoaxial joint. No significant abnormality involving the C1-C2 facet joints and the atlantooccipital articulations.

C2-C3: Preserved disc height and endplates. No disc herniation. Moderate left and mild right facet arthropathy. Mild left neural foraminal stenosis. Widely patent right neural foramen.

C3-C4: Preserved disc height and endplates. Mild facet arthropathy bilaterally. No disc herniation, spinal canal or neural foraminal stenosis.

C4-C5: Mild to moderate disc space narrowing and chronic endplate degenerative change with anterior disc bulge osteophyte complex and right greater than left uncovertebral ridging. Central disc protrusion-type herniation creating mild spinal canal stenosis compressing the cervical cord. Mild to moderate facet arthropathy bilaterally. Moderate bilateral neural foraminal stenosis worse on the right.

C5-C6: Moderate disc space narrowing. Moderate chronic endplate degenerative change with anterior disc bulge osteophyte complex and bilateral uncovertebral ridging. Broad-based posterior disc

protrusion creating mild to moderate spinal canal stenosis. Low density structure in the left neural foramen creating moderate to severe left neural foraminal stenosis may reflect uncovertebral ridging or disc herniation. Mild facet arthropathy bilaterally.

C6-C7: Preserved disc height. Mild facet arthropathy bilaterally. No disc herniation, spinal canal and neural foraminal stenosis.

C7-T1 and visualized upper thoracic levels are without significant abnormality.

IMPRESSION:
1. Motion degraded exam.
2. Flattening of cervical lordosis with scoliosis may reflect spasm/soft tissue injury.
3. No fracture or destructive process.
4. Multilevel degenerative changes, detailed above. This includes moderate neural foraminal stenosis bilaterally at C4-C5.
5. Moderate to severe left-sided neural foraminal stenosis at the C5-C6 level. This may be secondary to uncovertebral ridging or left foraminal disc protrusion-type herniation. This would be better evaluated with CT scan. If this represents a hernia, it may have been caused or worsened by trauma noted in the clinical history.
6. Central disc herniation at C4-C5 creating mild spinal canal stenosis. C5-C6 central disc herniation creating mild to moderate spinal canal stenosis. Assuming no other recent injury, there is a medical probability that either of these herniations is related to the patient's reported injury. Clinical correlation is recommended to confirm this.
7. Other findings above.

Report Electronically Signed by: Henley, M.D., David
Report Electronically Signed on: 3/8/2024 03:14 PM

Radiology Report from an IAC Accredited Facility

Iceberg Health Lawrenceville
1670 McKendree Church Road Suite 400B
Lawrenceville, GA 30043-4100
(678) 505-0000

Initial Exam

Patient Name: Francisca Diaz Alvarado
Date of Birth: 10/3/1969

Provider: Marita Thompson, DC
Date of Service: 3/1/2024

SUBJECTIVE

PRIMARY COMPLAINT
Right hip, Left hip

- Aching pain: 6/10
  Described as intermittent to frequent (up to between 50% and 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by lifting and changing positions; decreased by Prescribed Pain Medication.

SECOND COMPLAINT
Neck

- Pain: 7/10
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by changing positions; decreased by Prescribed Pain Medication.

THIRD COMPLAINT
Head

- Aching pain: 6/10
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by changing positions; decreased by Prescribed Pain Medication.

FOURTH COMPLAINT
Left shoulder, Right shoulder

- Aching pain: 7/10
  Described as frequent (up to 75% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by lifting, bending and changing positions; decreased by Prescribed Pain Medication.

accident happened on 2.20.24, she got rear ended, works putting together parts for air conditioning. neck shoulder and hi pain and gets headaches. gets random tingling feeling going down her legs that end up going numb for a little bit

OBJECTIVE

PALPATORY FINDINGS
Spinal asymmetry/fixation: C1; C4; T1; T2; T5; T10; Pelvis; SI.

Tenderness

Left Side

- Shoulder (moderate)
- Lower Back (moderate-severe)

Right Side

- Shoulder (moderate)
- Lower Back (moderate-severe)

Spasm/Hypertonicity

Left Side

- Cervical Spine (mild)
- Shoulder (mild)

Right Side

- Shoulder (mild)
- Thoracic Spine (moderate-severe)
- Lower Back (moderate-severe)

ORTHOPEDIC AND NEUROLOGICAL TESTS

Test	Left	Right	Comment
Cervical distraction [1]	Positive	Positive	
Foraminal compression test[2]	Positive	Positive	
Shoulder depression test[3]	Positive	Positive	
Nachlas test[4]	Positive	Positive	
Patrick test[5]	Positive	Positive	

[1]Relief of radicular pain suggests nerve root compression. Decreased local pain could indicate facet impingement. Increased local pain could indicate strain or sprain.

[2]Radicular pain could indicate pressure on the nerve root. Localized pain could indicate sprain or strain or foraminal encroachment.

[3]Pain in the arm could indicate adhesions of the nerve roots. Local pain could indicate sprain or strain or adhesions of the shoulder joint capsule.

[4]Sacroiliac, lumbosacral, or radiating pain may be indicative of a sacroiliac or lumbosacral disorder.

[5]Pain in the hip during abduction and external rotation may indicate a coxa pathologic condition.

RANGE OF MOTION

Cervical ROM - estimated

Test	Measurement	Limited By	Comment
Flexion	40° (80% of normal)		
Left lateral flexion	35° (77% of normal)	Pain	
Extension	45° (75% of normal)	Pain	
Right lateral flexion	35° (77% of normal)	Pain	
Left rotation	70° (87% of normal)	Pain	
Right rotation	70° (87% of normal)	Pain	

Lumbar ROM - estimated

Test	Measurement	Limited By	Comment
Flexion	45° (75% of normal)	Pain	
Extension	15° (60% of normal)	Pain	
Left lateral flexion	15° (60% of normal)	Pain	
Right ateral flexion	15° (60% of normal)	Pain	
Left rotation	25° (83% of normal)		
Right rotation	25° (83% of normal)		

DERMATOMES

Anterior

Location	Test	Result
Left-Lumbar	pinwheel-sensitivity	Paraesthesia
Right-Lumbar	pinwheel-sensitivity	Paraesthesia

ASSESSMENT

Stage of Care: Acute

DIAGNOSES

1. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)
2. Acute post-traumatic headache, intractable (G44.311)
3. Disorder of ligament, vertebrae (M24.28)
4. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
5. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
6. Strain of muscle, fascia and tendon at neck level, initial encounter (S16.1XXA)
7. Strain of muscle, fascia and tendon of lower back, initial encounter (S39.012A)
8. Cervicalgia (M54.2)
9. Low back pain, unspecified (M54.50)
10. Other muscle spasm (M62.838)
11. Segmental and somatic dysfunction of cervical region (M99.01)
12. Segmental and somatic dysfunction of thoracic region (M99.02)
13. Segmental and somatic dysfunction of lumbar region (M99.03)

LONG-TERM GOALS

- Be able to bend without having pain.
- Change positions without pain.
- Decrease headache pain.
- Be able to lift without pain.

PLAN

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of diversified technique was provided specifically to C1; C4; T1; T2; T5; T10; Pelvis; SI, to reduce pain reported at initial encounter by 25%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 25%, reduce muscle spasm, reduce chronic myofascial pain.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to Back, Neck, to reduce pain reported at initial encounter by 25%, decrease significant facet pain, break-up adhesions, reduce chronic myofascial pain.
- Ice/Heat (97010) consisting of hot pack was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 25%, reduce severity of palpable muscle spasm, reduce swelling/inflammation.
- Diagnostic Tests: X-ray cervical 4-5 views (72050) was provided due to significant traumatic mechanism of injury, visualize baseline spine curvature and disc spacing.
- Diagnostic Tests: X-ray lumbosacral 2-3 views (72100) was provided due to significant

traumatic mechanism of injury, visualize baseline spine curvature and disc spacing.

- New Patient evaluation and management service, 99203, was provided today.

Patient received care without incident.

(Electronic Signature)
Provider Name: Marita Thompson, DC
Date: 03/04/2024 09:09 AM

Iceberg Health Lawrenceville
1670 McKendree Church Road Suite 400B
Lawrenceville, GA 30043-4100
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Francisca Diaz Alvarado
Date of Birth: 10/3/1969

Provider: Marita Thompson, DC
Date of Service: 3/4/2024

SUBJECTIVE

PRIMARY COMPLAINT
Right hip, Left hip

- Aching pain: 6/10
  Described as intermittent to frequent (up to between 50% and 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by lifting and changing positions; decreased by Prescribed Pain Medication.

SECOND COMPLAINT
Neck

- Pain: 7/10
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by changing positions; decreased by Prescribed Pain Medication.

THIRD COMPLAINT
Head

- Aching pain: 6/10
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by changing positions; decreased by Prescribed Pain Medication.

FOURTH COMPLAINT
Left shoulder, Right shoulder

- Aching pain: 7/10
  Described as frequent (up to 75% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by lifting, bending and changing positions; decreased by Prescribed Pain Medication.

ROF done **MT**

OBJECTIVE

PALPATORY FINDINGS

Spinal asymmetry/fixation: C1; T1; T2; T5; T10; Pelvis; SI; **C5. No findings: C4**.

Tenderness

Left Side	Right Side

- Shoulder (Moderate)
- Lower Back (Moderate-Severe)

- Shoulder (Moderate)
- Lower Back (Moderate-Severe)

Spasm/Hypertonicity

Left Side

- Cervical Spine (Mild)
- Shoulder (Mild)

Right Side

- Shoulder (Mild)
- Thoracic Spine (Moderate-Severe)
- Lower Back (Moderate-Severe)

ASSESSMENT

Stage of Care: Acute

DIAGNOSES

1. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)
2. Acute post-traumatic headache, intractable (G44.311)
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10. Other muscle spasm (M62.838)
11. Segmental and somatic dysfunction of cervical region (M99.01)
12. Segmental and somatic dysfunction of thoracic region (M99.02)
13. Segmental and somatic dysfunction of lumbar region (M99.03)

CROFT GUIDELINES – Classifications of Cervical Acceleration/Deceleration (CAD) Trauma
Types of Collisions: (Occupant maybe driver or passenger. Criteria does not consider loss of consciousness, the use of seat belts, shoulder harnesses or other factors).
Type I Primary rear impact (struck car moving or stationary)**
Type II Primary side impact**
Type III Primary frontal impact**
Other: Multiple Impacts, Rollover, Spinout etc.
CAD Grades of Severity of Injury:**
Grade I: Minimal; No limitations of motion or ligamentous injury, No neurological findings
Grade II: Slight; Limitations of motion no ligamentous injury, no neurological findings
Grade III: Moderate; Limitations of motion; some ligamentous injury, neurological findings may be present.
Grade IV: Mod/severe; Limitations of motion; ligamentous instability; neurological findings present. Fracture or disc derangement
Grade V: Severe; requires surgical management/stabilization
Stages of Injury:**
Stage I: acute; inflammatory phase; 0 – 72 hours
Stage II: sub-acute; repair phase; 72 hours – 14 weeks post MVCOI
Stage III: remodeling phase; 14 weeks – 12 months or more post MVCOI
Stage IV: chronic; permanent injury
****MVCOI** Motor Vehicle Collision Occupant Injuries**

Protocol Frequency and Duration of Care in CAD Traumatology**

	5x/w	3x/w	2x/w	1x/w	1x/mos.	TD	TN
Grade I	**1w**	**1-2w**	**2-3w**	**<4w**	*****	**<11w**	**<21**
Grade II	**1w**	**<4w**	**<4w**	**<4w**	**<4mo.**	**<29w**	**<33**
Grade III	**1-2w**	**<10w**	**<10w**	**<10w**	**<6mos.**	**<56w**	**<76**
Grade IV	**2-3w**	**<16w**	**<12w**	**<20w**	**prn**	**prn**	**prn**

Grade V Surgical stabilization necessary–chiropractic care post-surgical

TD = treatment duration * Possible follow-up at one month

TN = treatment total number prn May require permanent monthly or prn (as needed) treatment

According to CROFT guidelines for cervical acceleration/deceleration, this patient is in the Stage 2 Injury Phase and has a Grade 3 severity of Injury. I will be putting him on a treatment plan based on CROFT protocols. My treatment plan will utilize different specialties to cut down the length of treatment and help the patient improve as efficiently as possible.

PLAN

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of diversified technique was provided specifically to C1; **C5**; T1; T2; T5; T10; Pelvis; SI, to reduce pain reported at initial encounter by 25%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 25%, reduce muscle spasm, reduce chronic myofascial pain.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to Back, Neck, to reduce pain reported at initial encounter by 25%, decrease significant facet pain, break-up adhesions, reduce chronic myofascial pain.
- Ice/Heat (97010) consisting of hot pack was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 25%, reduce severity of palpable muscle spasm, reduce swelling/inflammation.
- **Report of Findings (99212) was provided specifically to Neck, Head, Left Shoulder, Right Shoulder, Left Pelvis, Right Pelvis, Back, to enable patient to manage injury without re-aggravation.**

Patient received care without incident.

(Electronic Signature)
Provider Name: Marita Thompson, DC
Date: 03/04/2024 03:19 PM

Iceberg Health Lawrenceville
1670 McKendree Church Road Suite 400B
Lawrenceville, GA 30043-4100
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Francisca Diaz Alvarado
Date of Birth: 10/3/1969

Provider: Rita Patel, DC
Date of Service: 3/6/2024

SUBJECTIVE

PRIMARY COMPLAINT
Right hip, Left hip

- Aching pain: 6/10
  Described as intermittent to frequent (up to between 50% and 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by lifting and changing positions; decreased by Prescribed Pain Medication.

SECOND COMPLAINT
Neck

- Pain: 7/10
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by changing positions; decreased by Prescribed Pain Medication.

THIRD COMPLAINT
Head

- Aching pain: 6/10
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by changing positions; decreased by Prescribed Pain Medication.

FOURTH COMPLAINT
Left shoulder, Right shoulder

- Aching pain: 7/10
  Described as frequent (up to 75% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by lifting, bending and changing positions; decreased by Prescribed Pain Medication.

OBJECTIVE

PALPATORY FINDINGS
Spinal asymmetry/fixation: C1; C5; T1; T2; T5; T10; Pelvis; SI.
Tenderness

Left Side	Right Side

- Shoulder (Moderate)
- Lower Back (Moderate-Severe)

- Shoulder (Moderate)
- Lower Back (Moderate-Severe)

Spasm/Hypertonicity

Left Side

- Cervical Spine (Mild)
- Shoulder (Mild)

Right Side

- Shoulder (Mild)
- Thoracic Spine (Moderate-Severe)
- Lower Back (Moderate-Severe)

ASSESSMENT

Stage of Care: Acute

DIAGNOSES

1. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)
2. Acute post-traumatic headache, intractable (G44.311)
3. Disorder of ligament, vertebrae (M24.28)
4. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
5. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
6. Strain of muscle, fascia and tendon at neck level, initial encounter (S16.1XXA)
7. Strain of muscle, fascia and tendon of lower back, initial encounter (S39.012A)
8. Cervicalgia (M54.2)
9. Low back pain, unspecified (M54.50)
10. Other muscle spasm (M62.838)
11. Segmental and somatic dysfunction of cervical region (M99.01)
12. Segmental and somatic dysfunction of thoracic region (M99.02)
13. Segmental and somatic dysfunction of lumbar region (M99.03)

PLAN

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of diversified technique was provided specifically to C1; C5; T1; T2; T5; T10; Pelvis; SI, to reduce pain reported at initial encounter by 25%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 25%, reduce muscle spasm, reduce chronic myofascial pain.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to Back, Neck, to reduce pain reported at initial encounter by 25%, decrease significant facet pain, break-up adhesions, reduce chronic myofascial pain.
- Ice/Heat (97010) consisting of hot pack was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 25%, reduce severity of palpable muscle spasm, reduce swelling/inflammation.

Patient received care without incident.

(Electronic Signature)
Provider Name: Rita Patel, DC
Date: 03/06/2024 10:54 AM

Iceberg Health Lawrenceville
1670 McKendree Church Road Suite 400B
Lawrenceville, GA 30043-4100
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Francisca Diaz Alvarado
Date of Birth: 10/3/1969

Provider: Marita Thompson, DC
Date of Service: 3/8/2024

SUBJECTIVE

PRIMARY COMPLAINT
Right hip, Left hip

- Aching pain: 6/10
  Described as intermittent to frequent (up to between 50% and 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by lifting and changing positions; decreased by Prescribed Pain Medication.

SECOND COMPLAINT
Neck

- Pain: 7/10
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by changing positions; decreased by Prescribed Pain Medication.

THIRD COMPLAINT
Head

- Aching pain: 6/10
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by changing positions; decreased by Prescribed Pain Medication.

FOURTH COMPLAINT
Left shoulder, Right shoulder

- Aching pain: 7/10
  Described as frequent (up to 75% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by lifting, bending and changing positions; decreased by Prescribed Pain Medication.

OBJECTIVE

PALPATORY FINDINGS

Spinal asymmetry/fixation: C1; C5; T1; T2; T5; T10; Pelvis; **L3; L5. No findings: SI**.

Tenderness

Left Side	Right Side

- Shoulder (Moderate)
- Lower Back (Moderate-Severe)

- Shoulder (Moderate)
- Lower Back (Moderate-Severe)

Spasm/Hypertonicity

Left Side

- Cervical Spine (Mild)
- Shoulder (Mild)

Right Side

- Shoulder (Mild)
- Thoracic Spine (Moderate-Severe)
- Lower Back (Moderate-Severe)

ASSESSMENT

Stage of Care: Acute

DIAGNOSES

1. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)
2. Acute post-traumatic headache, intractable (G44.311)
3. Disorder of ligament, vertebrae (M24.28)
4. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
5. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
6. Strain of muscle, fascia and tendon at neck level, initial encounter (S16.1XXA)
7. Strain of muscle, fascia and tendon of lower back, initial encounter (S39.012A)
8. Cervicalgia (M54.2)
9. Low back pain, unspecified (M54.50)
10. Other muscle spasm (M62.838)
11. Segmental and somatic dysfunction of cervical region (M99.01)
12. Segmental and somatic dysfunction of thoracic region (M99.02)
13. Segmental and somatic dysfunction of lumbar region (M99.03)

PLAN

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of diversified technique was provided specifically to C1; C5; T1; T2; T5; T10; **L3**; **L5**; Pelvis, to reduce pain reported at initial encounter by 25%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 25%, reduce muscle spasm, reduce chronic myofascial pain.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to Back, Neck, to reduce pain reported at initial encounter by 25%, decrease significant facet pain, break-up adhesions, reduce chronic myofascial pain.
- Ice/Heat (97010) consisting of hot pack was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 25%, reduce severity of palpable muscle spasm, reduce swelling/inflammation.
- **Manual Therapy (Ea 15 Min) (97140) consisting of trigger point therapy was provided specifically to Neck, Right Shoulder, Left Shoulder for 10 minutes (1 unit), to reduce pain reported at initial encounter by 25%, increase pain free range of motion by 25% from initial exam, reduce muscle spasm, break-up adhesions, reduce chronic myofascial pain.**

Patient received care without incident.

(Electronic Signature)
Provider Name: Marita Thompson, DC
Date: 03/08/2024 11:09 AM

Iceberg Health Lawrenceville
1670 McKendree Church Road Suite 400B
Lawrenceville, GA 30043-4100
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Francisca Diaz Alvarado
Date of Birth: 10/3/1969

Provider: Marita Thompson, DC
Date of Service: 3/11/2024

SUBJECTIVE

PRIMARY COMPLAINT
Right hip, Left hip

- Aching pain: 6/10
  Described as intermittent to frequent (up to between 50% and 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by lifting and changing positions; decreased by Prescribed Pain Medication.

SECOND COMPLAINT
Neck

- Pain: 7/10
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by changing positions; decreased by Prescribed Pain Medication.

THIRD COMPLAINT
Head

- Aching pain: 6/10
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by changing positions; decreased by Prescribed Pain Medication.

FOURTH COMPLAINT
Left shoulder, Right shoulder

- Aching pain: 7/10
  Described as frequent (up to 75% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by lifting, bending and changing positions; decreased by Prescribed Pain Medication.

OBJECTIVE

PALPATORY FINDINGS

Spinal asymmetry/fixation: C1; C5; T1; T5; T10; L5; Pelvis; **T12; L4. No findings: T2; L3**.

Tenderness

Left Side	Right Side

- Shoulder (Moderate)
- Lower Back (Moderate-Severe)

- Shoulder (Moderate)
- Lower Back (Moderate-Severe)

Spasm/Hypertonicity

Left Side

- Cervical Spine (Mild)
- Shoulder (Mild)

Right Side

- Shoulder (Mild)
- Thoracic Spine (Moderate-Severe)
- Lower Back (Moderate-Severe)

ASSESSMENT

Stage of Care: Acute

DIAGNOSES

1. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)
2. Acute post-traumatic headache, intractable (G44.311)
3. Disorder of ligament, vertebrae (M24.28)
4. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
5. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
6. Strain of muscle, fascia and tendon at neck level, initial encounter (S16.1XXA)
7. Strain of muscle, fascia and tendon of lower back, initial encounter (S39.012A)
8. Cervicalgia (M54.2)
9. Low back pain, unspecified (M54.50)
10. Other muscle spasm (M62.838)
11. Segmental and somatic dysfunction of cervical region (M99.01)
12. Segmental and somatic dysfunction of thoracic region (M99.02)
13. Segmental and somatic dysfunction of lumbar region (M99.03)

PLAN

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of **activator technique**, **Thompson technique** was provided specifically to C1; C5; T1; T5; T10; **T12**; **L4**; L5; Pelvis, to reduce pain reported at initial encounter by 25%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 25%, reduce muscle spasm, reduce chronic myofascial pain.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to Back, Neck, to reduce pain reported at initial encounter by 25%, decrease significant facet pain, break-up adhesions, reduce chronic myofascial pain.
- Ice/Heat (97010) consisting of hot pack was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 25%, reduce severity of palpable muscle spasm, reduce swelling/inflammation.

Patient received care without incident.

(Electronic Signature)
Provider Name: Marita Thompson, DC

Date: 03/11/2024 03:18 PM

Iceberg Health Lawrenceville
1670 McKendree Church Road Suite 400B
Lawrenceville, GA 30043-4100
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Francisca Diaz Alvarado
Date of Birth: 10/3/1969

Provider: Marita Thompson, DC
Date of Service: 3/14/2024

SUBJECTIVE

PRIMARY COMPLAINT
Right hip, Left hip

- Aching pain: 6/10
  Described as intermittent to frequent (up to between 50% and 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by lifting and changing positions; decreased by Prescribed Pain Medication.

SECOND COMPLAINT
Neck

- Pain: 7/10
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by changing positions; decreased by Prescribed Pain Medication.

THIRD COMPLAINT
Head

- Aching pain: 6/10
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by changing positions; decreased by Prescribed Pain Medication.

FOURTH COMPLAINT
Left shoulder, Right shoulder

- Aching pain: 7/10
  Described as frequent (up to 75% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by lifting, bending and changing positions; decreased by Prescribed Pain Medication.

OBJECTIVE

PALPATORY FINDINGS

Spinal asymmetry/fixation: C1; C5; T1; T5; T12; L5; Pelvis; **SI. No findings: T10; L4**.

Tenderness

Left Side	Right Side

- Shoulder (Moderate)
- Lower Back (Moderate-Severe)

- Shoulder (Moderate)
- Lower Back (Moderate-Severe)

Spasm/Hypertonicity

Left Side

- Cervical Spine (Mild)
- Shoulder (Mild)

Right Side

- Shoulder (Mild)
- Thoracic Spine (Moderate-Severe)
- Lower Back (Moderate-Severe)

ASSESSMENT

Stage of Care: Acute

DIAGNOSES

1. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)
2. Acute post-traumatic headache, intractable (G44.311)
3. Disorder of ligament, vertebrae (M24.28)
4. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
5. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
6. Strain of muscle, fascia and tendon at neck level, initial encounter (S16.1XXA)
7. Strain of muscle, fascia and tendon of lower back, initial encounter (S39.012A)
8. Cervicalgia (M54.2)
9. Low back pain, unspecified (M54.50)
10. Other muscle spasm (M62.838)
11. Segmental and somatic dysfunction of cervical region (M99.01)
12. Segmental and somatic dysfunction of thoracic region (M99.02)
13. Segmental and somatic dysfunction of lumbar region (M99.03)

PLAN

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of activator technique, Thompson technique was provided specifically to C1; C5; T1; T5; T12; L5; Pelvis; **SI**, to reduce pain reported at initial encounter by 25%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 25%, reduce muscle spasm, reduce chronic myofascial pain.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to Back, Neck, to reduce pain reported at initial encounter by 25%, decrease significant facet pain, break-up adhesions, reduce chronic myofascial pain.

Patient received care without incident.

(Electronic Signature)
Provider Name: Marita Thompson, DC
Date: 03/14/2024 10:42 AM

Iceberg Health Lawrenceville
1670 McKendree Church Road Suite 400B
Lawrenceville, GA 30043-4100
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Francisca Diaz Alvarado
Date of Birth: 10/3/1969

Provider: Marita Thompson, DC
Date of Service: 3/18/2024

SUBJECTIVE

PRIMARY COMPLAINT
Right hip, Left hip

- Aching pain: 6/10
  Described as intermittent to frequent (up to between 50% and 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by lifting and changing positions; decreased by Prescribed Pain Medication.

SECOND COMPLAINT
Neck

- Pain: 7/10
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by changing positions; decreased by Prescribed Pain Medication.

THIRD COMPLAINT
Head

- Aching pain: 6/10
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by changing positions; decreased by Prescribed Pain Medication.

FOURTH COMPLAINT
Left shoulder, Right shoulder

- Aching pain: 7/10
  Described as frequent (up to 75% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by lifting, bending and changing positions; decreased by Prescribed Pain Medication.

OBJECTIVE

PALPATORY FINDINGS

Spinal asymmetry/fixation: C5; T1; T5; Pelvis; SI; **C2; T10; L2. No findings: C1; T12; L5**.

Tenderness

Left Side | Right Side

- Shoulder (Moderate)
- Lower Back (Moderate-Severe)

- Shoulder (Moderate)
- Lower Back (Moderate-Severe)

Spasm/Hypertonicity

Left Side

- Cervical Spine (Mild)
- Shoulder (Mild)

Right Side

- Shoulder (Mild)
- Thoracic Spine (Moderate-Severe)
- Lower Back (Moderate-Severe)

ASSESSMENT

Stage of Care: Acute

DIAGNOSES

1. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)
2. Acute post-traumatic headache, intractable (G44.311)
3. Disorder of ligament, vertebrae (M24.28)
4. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
5. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
6. Strain of muscle, fascia and tendon at neck level, initial encounter (S16.1XXA)
7. Strain of muscle, fascia and tendon of lower back, initial encounter (S39.012A)
8. Cervicalgia (M54.2)
9. Low back pain, unspecified (M54.50)
10. Other muscle spasm (M62.838)
11. Segmental and somatic dysfunction of cervical region (M99.01)
12. Segmental and somatic dysfunction of thoracic region (M99.02)
13. Segmental and somatic dysfunction of lumbar region (M99.03)

PLAN

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of activator technique, Thompson technique was provided specifically to **C2**; C5; T1; T5; **T10**; **L2**; Pelvis; SI, to reduce pain reported at initial encounter by 25%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 25%, reduce muscle spasm, reduce chronic myofascial pain.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to Back, Neck, to reduce pain reported at initial encounter by 25%, decrease significant facet pain, break-up adhesions, reduce chronic myofascial pain.

Patient received care without incident.

(Electronic Signature)
Provider Name: Marita Thompson, DC
Date: 03/18/2024 03:19 PM

Iceberg Health Lawrenceville
1670 McKendree Church Road Suite 400B
Lawrenceville, GA 30043-4100
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Francisca Diaz Alvarado
Date of Birth: 10/3/1969

Provider: Marita Thompson, DC
Date of Service: 3/20/2024

SUBJECTIVE

PRIMARY COMPLAINT
Right hip, Left hip

- Aching pain: 6/10
  Described as intermittent to frequent (up to between 50% and 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by lifting and changing positions; decreased by Prescribed Pain Medication.

SECOND COMPLAINT
Neck

- Pain: 7/10
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by changing positions; decreased by Prescribed Pain Medication.

THIRD COMPLAINT
Head

- Aching pain: 6/10
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by changing positions; decreased by Prescribed Pain Medication.

FOURTH COMPLAINT
Left shoulder, Right shoulder

- Aching pain: 7/10
  Described as frequent (up to 75% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by lifting, bending and changing positions; decreased by Prescribed Pain Medication.

OBJECTIVE

PALPATORY FINDINGS

Spinal asymmetry/fixation: T1; T5; T10; Pelvis; SI; **C1; C4; T8. No findings: C2; C5; L2**.

Tenderness

Left Side	Right Side

- Shoulder (Moderate)
- Lower Back (Moderate-Severe)

- Shoulder (Moderate)
- Lower Back (Moderate-Severe)

Spasm/Hypertonicity

Left Side

- Cervical Spine (Mild)
- Shoulder (Mild)

Right Side

- Shoulder (Mild)
- Thoracic Spine (Moderate-Severe)
- Lower Back (Moderate-Severe)

ASSESSMENT

Stage of Care: Acute

DIAGNOSES

1. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)
2. Acute post-traumatic headache, intractable (G44.311)
3. Disorder of ligament, vertebrae (M24.28)
4. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
5. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
6. Strain of muscle, fascia and tendon at neck level, initial encounter (S16.1XXA)
7. Strain of muscle, fascia and tendon of lower back, initial encounter (S39.012A)
8. Cervicalgia (M54.2)
9. Low back pain, unspecified (M54.50)
10. Other muscle spasm (M62.838)
11. Segmental and somatic dysfunction of cervical region (M99.01)
12. Segmental and somatic dysfunction of thoracic region (M99.02)
13. Segmental and somatic dysfunction of lumbar region (M99.03)

PLAN

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of activator technique, Thompson technique was provided specifically to **C1**; **C4**; T1; T5; **T8**; **T10**; Pelvis; SI, to reduce pain reported at initial encounter by 25%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 25%, reduce muscle spasm, reduce chronic myofascial pain.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to Back, Neck, to reduce pain reported at initial encounter by 25%, decrease significant facet pain, break-up adhesions, reduce chronic myofascial pain.

Patient received care without incident.

(Electronic Signature)
Provider Name: Marita Thompson, DC
Date: 03/20/2024 03:26 PM

Iceberg Health Lawrenceville
1670 McKendree Church Road Suite 400B
Lawrenceville, GA 30043-4100
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Exam

Patient Name: Francisca Diaz Alvarado
Date of Birth: 10/3/1969

Provider: Marita Thompson, DC
Date of Service: 3/22/2024

SUBJECTIVE

PRIMARY COMPLAINT
Right hip, Left hip

- Aching pain: 6/10
  Described as intermittent to frequent (up to between 50% and 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by lifting, **walking** and changing positions; decreased by Prescribed Pain Medication.

SECOND COMPLAINT
Neck

- Pain: **From 7 to 5/10 (20% decrease)**
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by changing positions; decreased by Prescribed Pain Medication.

THIRD COMPLAINT
Head

- Aching pain: **From 6 to 4/10 (20% decrease)**
  Described as **intermittent (up to 50% of day)** of moderate intensity (requires significant or complete modification to ADL); increased by changing positions; decreased by Prescribed Pain Medication.

FOURTH COMPLAINT
Left shoulder, Right shoulder

- Aching pain: **From 7 to 4/10 (30% decrease)**
  Described as frequent (up to 75% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by lifting, bending and changing positions; decreased by Prescribed Pain Medication.

pt had reeaxam today with some improvement in all areas

OBJECTIVE

PALPATORY FINDINGS

Spinal asymmetry/fixation: C1; C4; T1; T5; T8; Pelvis; SI**. No findings: T10**.

Tenderness

Left Side	Right Side

- Lower Back **(resolved)**
- Shoulder **(decreased 5-10% to mild-moderate)**

- Lower Back **(resolved)**
- Shoulder **(decreased 5-10% to mild-moderate)**

Spasm/Hypertonicity

Left Side

- Cervical Spine **(decreased 5-10% to minimal)**
- Shoulder (Mild)

Right Side

- Shoulder **(decreased 10-20%)**
- Thoracic Spine **(decreased 35-50% to minimal)**
- Lower Back **(decreased 35-50% to minimal)**

ORTHOPEDIC AND NEUROLOGICAL TESTS

Test	Left	Right	Comment
Cervical distraction [1]	**WNL**	Positive	
Foraminal compression test[2]	Positive	Positive	
Shoulder depression test[3]	**WNL**	Positive	
Nachlas test[4]	Positive	Positive	
Patrick test[5]	Positive	Positive	

[1]Relief of radicular pain suggests nerve root compression. Decreased local pain could indicate facet impingement. Increased local pain could indicate strain or sprain.

[2]Radicular pain could indicate pressure on the nerve root. Localized pain could indicate sprain or strain or foraminal encroachment.

[3]Pain in the arm could indicate adhesions of the nerve roots. Local pain could indicate sprain or strain or adhesions of the shoulder joint capsule.

[4]Sacroiliac, lumbosacral, or radiating pain may be indicative of a sacroiliac or lumbosacral disorder.

[5]Pain in the hip during abduction and external rotation may indicate a coxa pathologic condition.

RANGE OF MOTION

Cervical ROM - estimated

Test	Measurement	Limited By	Comment
Flexion	40° (80% of normal)		
Left lateral flexion	35° (77% of normal)	Pain	
Extension	45° (75% of normal)	Pain	
Right lateral flexion	35° (77% of normal)	Pain	
Left rotation	70° (87% of normal)		
Right rotation	70° (87% of normal)		

Lumbar ROM - estimated

Test	Measurement	Limited By	Comment
Flexion	45° (75% of normal)	Pain	
Extension	15° (60% of normal)	Pain	
Left lateral flexion	15° (60% of normal)		
Right ateral flexion	15° (60% of normal)	Pain	

Left rotation	25° (83% of normal)	**Pain**	
Right rotation	25° (83% of normal)		

DERMATOMES

Anterior

Location	Test	Result
Left-Lumbar	pinwheel-sensitivity	Paraesthesia
Right-Lumbar	pinwheel-sensitivity	Paraesthesia

ASSESSMENT

Stage of Care: Rehabilitative

DIAGNOSES

1. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)
2. Acute post-traumatic headache, intractable (G44.311)
3. Disorder of ligament, vertebrae (M24.28)
4. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
5. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
6. Strain of muscle, fascia and tendon at neck level, initial encounter (S16.1XXA)
7. Strain of muscle, fascia and tendon of lower back, initial encounter (S39.012A)
8. Cervicalgia (M54.2)
9. Low back pain, unspecified (M54.50)
10. Other muscle spasm (M62.838)
11. Segmental and somatic dysfunction of cervical region (M99.01)
12. Segmental and somatic dysfunction of thoracic region (M99.02)
13. Segmental and somatic dysfunction of lumbar region (M99.03)

PLAN

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of activator technique, Thompson technique was provided specifically to C1; C4; T1; T5; T8; Pelvis; SI, to reduce pain reported at initial encounter by 25%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 25%, reduce muscle spasm, reduce chronic myofascial pain.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to Back, Neck, to reduce pain reported at initial encounter by 25%, decrease significant facet pain, break-up adhesions, reduce chronic myofascial pain.
- **Ice/Heat (97010) consisting of hot pack was provided specifically to Neck, Left Scapula, Left Rib Cage, Right Scapula, Right Rib Cage, Back, to reduce pain reported at initial encounter by 25%, reduce muscle spasm, reduce swelling/inflammation.**

Patient received care without incident.

(Electronic Signature)
Provider Name: Marita Thompson, DC
Date: 03/22/2024 03:15 PM

Iceberg Health Lawrenceville
1670 McKendree Church Road Suite 400B
Lawrenceville, GA 30043-4100
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Francisca Diaz Alvarado
Date of Birth: 10/3/1969

Provider: Marita Thompson, DC
Date of Service: 3/25/2024

SUBJECTIVE

PRIMARY COMPLAINT
Right hip, Left hip

- Aching pain: 6/10
  Described as intermittent to frequent (up to between 50% and 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by lifting, walking and changing positions; decreased by Prescribed Pain Medication.

SECOND COMPLAINT
Neck

- Pain: 5/10
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by changing positions; decreased by Prescribed Pain Medication.

THIRD COMPLAINT
Head

- Aching pain: 4/10
  Described as intermittent (up to 50% of day) of moderate intensity (requires significant or complete modification to ADL); increased by changing positions; decreased by Prescribed Pain Medication.

FOURTH COMPLAINT
Left shoulder, Right shoulder

- Aching pain: 4/10
  Described as frequent (up to 75% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by lifting, bending and changing positions; decreased by Prescribed Pain Medication.

OBJECTIVE

PALPATORY FINDINGS

Spinal asymmetry/fixation: C1; C4; T1; T5; T8; Pelvis; SI; **T12.**

Tenderness

Left Side	Right Side

- Shoulder (Mild-Moderate)
- Shoulder (Mild-Moderate)

Spasm/Hypertonicity

Left Side

- Cervical Spine (Minimal)
- Shoulder (Mild)

Right Side

- Shoulder
- Thoracic Spine (Minimal)
- Lower Back (Minimal)

ASSESSMENT

Stage of Care: Rehabilitative

DIAGNOSES

1. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)
2. Acute post-traumatic headache, intractable (G44.311)
3. Disorder of ligament, vertebrae (M24.28)
4. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
5. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
6. Strain of muscle, fascia and tendon at neck level, initial encounter (S16.1XXA)
7. Strain of muscle, fascia and tendon of lower back, initial encounter (S39.012A)
8. Cervicalgia (M54.2)
9. Low back pain, unspecified (M54.50)
10. Other muscle spasm (M62.838)
11. Segmental and somatic dysfunction of cervical region (M99.01)
12. Segmental and somatic dysfunction of thoracic region (M99.02)
13. Segmental and somatic dysfunction of lumbar region (M99.03)

PLAN

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of activator technique, Thompson technique was provided specifically to C1; C4; T1; T5; T8; **T12**; Pelvis; SI, to reduce pain reported at initial encounter by 25%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 25%, reduce muscle spasm, reduce chronic myofascial pain.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to Back, Neck, to reduce pain reported at initial encounter by 25%, decrease significant facet pain, break-up adhesions, reduce chronic myofascial pain.
- Ice/Heat (97010) consisting of hot pack was provided specifically to Neck, Left Scapula, Left Rib Cage, Right Scapula, Right Rib Cage, Back, to reduce pain reported at initial encounter by 25%, reduce muscle spasm, reduce swelling/inflammation.

Patient received care without incident.

(Electronic Signature)
Provider Name: Marita Thompson, DC
Date: 03/25/2024 03:10 PM

Addenda

MRI Results were reviewed today with the patient. The patient had the opportunity to ask questions and understand the findings. Due to positive findings, the patient will be referred to a specialist for further assessment.

Electronic Signature: Thompson, Marita DC

Added on Mar 25, 2024

Iceberg Health Lawrenceville
1670 McKendree Church Road Suite 400B
Lawrenceville, GA 30043-4100
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Francisca Diaz Alvarado
Date of Birth: 10/3/1969

Provider: Marita Thompson, DC
Date of Service: 3/27/2024

SUBJECTIVE

PRIMARY COMPLAINT
Right hip, Left hip

- Aching pain: 6/10
  Described as intermittent to frequent (up to between 50% and 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by lifting, walking and changing positions; decreased by Prescribed Pain Medication.

SECOND COMPLAINT
Neck

- Pain: 5/10
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by changing positions; decreased by Prescribed Pain Medication.

THIRD COMPLAINT
Head

- Aching pain: 4/10
  Described as intermittent (up to 50% of day) of moderate intensity (requires significant or complete modification to ADL); increased by changing positions; decreased by Prescribed Pain Medication.

FOURTH COMPLAINT
Left shoulder, Right shoulder

- Aching pain: 4/10
  Described as frequent (up to 75% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by lifting, bending and changing positions; decreased by Prescribed Pain Medication.

OBJECTIVE

PALPATORY FINDINGS

Spinal asymmetry/fixation: C1; C4; T5; T8; T12; Pelvis; SI**. No findings: T1**.

Tenderness

Left Side	Right Side

- Shoulder (Mild-Moderate)
- Shoulder (Mild-Moderate)

Spasm/Hypertonicity

Left Side

- Cervical Spine (Minimal)
- Shoulder (Mild)

Right Side

- Shoulder
- Thoracic Spine (Minimal)
- Lower Back (Minimal)

ASSESSMENT

Stage of Care: Rehabilitative

DIAGNOSES

1. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)
2. Acute post-traumatic headache, intractable (G44.311)
3. Disorder of ligament, vertebrae (M24.28)
4. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
5. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
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7. Strain of muscle, fascia and tendon of lower back, initial encounter (S39.012A)
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9. Low back pain, unspecified (M54.50)
10. Other muscle spasm (M62.838)
11. Segmental and somatic dysfunction of cervical region (M99.01)
12. Segmental and somatic dysfunction of thoracic region (M99.02)
13. Segmental and somatic dysfunction of lumbar region (M99.03)

PLAN

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of activator technique, Thompson technique was provided specifically to C1; C4; T5; T8; T12; Pelvis; SI, to reduce pain reported at initial encounter by 25%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 25%, reduce muscle spasm, reduce chronic myofascial pain.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to Back, Neck, to reduce pain reported at initial encounter by 25%, decrease significant facet pain, break-up adhesions, reduce chronic myofascial pain.
- Ice/Heat (97010) consisting of hot pack was provided specifically to Neck, Left Scapula, Left Rib Cage, Right Scapula, Right Rib Cage, Back, to reduce pain reported at initial encounter by 25%, reduce muscle spasm, reduce swelling/inflammation.

Patient received care without incident.

(Electronic Signature)
Provider Name: Marita Thompson, DC
Date: 03/27/2024 03:03 PM

Iceberg Health Lawrenceville
1670 McKendree Church Road Suite 400B
Lawrenceville, GA 30043-4100
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Francisca Diaz Alvarado
Date of Birth: 10/3/1969

Provider: Marita Thompson, DC
Date of Service: 3/29/2024

SUBJECTIVE

PRIMARY COMPLAINT
Right hip, Left hip

- Aching pain: 6/10
  Described as intermittent to frequent (up to between 50% and 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by lifting, walking and changing positions; decreased by Prescribed Pain Medication.

SECOND COMPLAINT
Neck

- Pain: 5/10
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by changing positions; decreased by Prescribed Pain Medication.

THIRD COMPLAINT
Head

- Aching pain: 4/10
  Described as intermittent (up to 50% of day) of moderate intensity (requires significant or complete modification to ADL); increased by changing positions; decreased by Prescribed Pain Medication.

FOURTH COMPLAINT
Left shoulder, Right shoulder

- Aching pain: 4/10
  Described as frequent (up to 75% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by lifting, bending and changing positions; decreased by Prescribed Pain Medication.

pt says she wakes up with numbness down her R arm. she has to toss and turn a lot through the night due to the neck and shoulder pain.

OBJECTIVE

PALPATORY FINDINGS
Spinal asymmetry/fixation: C1; T5; T8; T12; Pelvis; SI; **C3; L1. No findings: C4**.

Tenderness

Left Side

- Shoulder (Mild-Moderate)

Right Side

- Shoulder (Mild-Moderate)

Spasm/Hypertonicity

Left Side

- Cervical Spine (Minimal)
- Shoulder (Mild)

Right Side

- Shoulder
- Thoracic Spine (Minimal)
- Lower Back (Minimal)

ASSESSMENT

Stage of Care: Rehabilitative

DIAGNOSES

1. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)
2. Acute post-traumatic headache, intractable (G44.311)
3. Disorder of ligament, vertebrae (M24.28)
4. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
5. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
6. Strain of muscle, fascia and tendon at neck level, initial encounter (S16.1XXA)
7. Strain of muscle, fascia and tendon of lower back, initial encounter (S39.012A)
8. Cervicalgia (M54.2)
9. Low back pain, unspecified (M54.50)
10. Other muscle spasm (M62.838)
11. Segmental and somatic dysfunction of cervical region (M99.01)
12. Segmental and somatic dysfunction of thoracic region (M99.02)
13. Segmental and somatic dysfunction of lumbar region (M99.03)

PLAN

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of activator technique, Thompson technique was provided specifically to C1; **C3**; T5; T8; T12; **L1**; Pelvis; SI, to reduce pain reported at initial encounter by 25%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 25%, reduce muscle spasm, reduce chronic myofascial pain.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to Back, Neck, to reduce pain reported at initial encounter by 25%, decrease significant facet pain, break-up adhesions, reduce chronic myofascial pain.
- Ice/Heat (97010) consisting of hot pack was provided specifically to Neck, Left Scapula, Left Rib Cage, Right Scapula, Right Rib Cage, Back, to reduce pain reported at initial encounter by 25%, reduce muscle spasm, reduce swelling/inflammation.

Patient received care without incident.

(Electronic Signature)

Provider Name: Marita Thompson, DC
Date: 03/29/2024 04:32 PM

Iceberg Health Lawrenceville
1670 McKendree Church Road Suite 400B
Lawrenceville, GA 30043-4100
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Francisca Diaz Alvarado
Date of Birth: 10/3/1969

Provider: Rita Patel, DC
Date of Service: 4/1/2024

SUBJECTIVE

PRIMARY COMPLAINT
Right hip, Left hip

- Aching pain: 6/10
  Described as intermittent to frequent (up to between 50% and 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by lifting, walking and changing positions; decreased by Prescribed Pain Medication.

SECOND COMPLAINT
Neck

- Pain: 5/10
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by changing positions; decreased by Prescribed Pain Medication.

THIRD COMPLAINT
Head

- Aching pain: 4/10
  Described as intermittent (up to 50% of day) of moderate intensity (requires significant or complete modification to ADL); increased by changing positions; decreased by Prescribed Pain Medication.

FOURTH COMPLAINT
Left shoulder, Right shoulder

- Aching pain: 4/10
  Described as frequent (up to 75% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by lifting, bending and changing positions; decreased by Prescribed Pain Medication.

OBJECTIVE

PALPATORY FINDINGS
Spinal asymmetry/fixation: C1; C3; T5; T8; T12; L1; Pelvis; SI.
Tenderness

Left Side	Right Side

- Shoulder (Mild-Moderate)
- Shoulder (Mild-Moderate)

Spasm/Hypertonicity

Left Side

- Cervical Spine (Minimal)
- Shoulder (Mild)

Right Side

- Shoulder
- Thoracic Spine (Minimal)
- Lower Back (Minimal)

ASSESSMENT

Stage of Care: Rehabilitative

DIAGNOSES

1. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)
2. Acute post-traumatic headache, intractable (G44.311)
3. Disorder of ligament, vertebrae (M24.28)
4. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
5. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
6. Strain of muscle, fascia and tendon at neck level, initial encounter (S16.1XXA)
7. Strain of muscle, fascia and tendon of lower back, initial encounter (S39.012A)
8. Cervicalgia (M54.2)
9. Low back pain, unspecified (M54.50)
10. Other muscle spasm (M62.838)
11. Segmental and somatic dysfunction of cervical region (M99.01)
12. Segmental and somatic dysfunction of thoracic region (M99.02)
13. Segmental and somatic dysfunction of lumbar region (M99.03)

PLAN

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of activator technique, Thompson technique was provided specifically to C1; C3; T5; T8; T12; L1; Pelvis; SI, to reduce pain reported at initial encounter by 25%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 25%, reduce muscle spasm, reduce chronic myofascial pain.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to Back, Neck, to reduce pain reported at initial encounter by 25%, decrease significant facet pain, break-up adhesions, reduce chronic myofascial pain.
- Ice/Heat (97010) consisting of hot pack was provided specifically to Neck, Left Scapula, Left Rib Cage, Right Scapula, Right Rib Cage, Back, to reduce pain reported at initial encounter by 25%, reduce muscle spasm, reduce swelling/inflammation.
- **Therapeutic Exercise (Ea. 15 Min) (97110) consisting of Isometric muscle contraction exercises was provided specifically to Left Heel, Right Heel, Left Ankle, Right Ankle, Left Lower Leg, Right Lower Leg, Right Knee, Left Knee, Left Upper Leg, Right Upper Leg, Left Gluteals, Right Gluteals, Right Pelvis, Left Pelvis, Sacrum, Back for 15 minutes (1 unit), to increase pain free range of motion by 25% from initial exam.**

Patient received care without incident.

(Electronic Signature)
Provider Name: Rita Patel, DC
Date: 04/01/2024 03:35 PM

Iceberg Health Lawrenceville
1670 McKendree Church Road Suite 400B
Lawrenceville, GA 30043-4100
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Francisca Diaz Alvarado
Date of Birth: 10/3/1969

Provider: Marita Thompson, DC
Date of Service: 4/3/2024

SUBJECTIVE

PRIMARY COMPLAINT
Right hip, Left hip

- Aching pain: 6/10
  Described as intermittent to frequent (up to between 50% and 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by lifting, walking and changing positions; decreased by Prescribed Pain Medication.

SECOND COMPLAINT
Neck

- Pain: 5/10
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by changing positions; decreased by Prescribed Pain Medication.

THIRD COMPLAINT
Head

- Aching pain: 4/10
  Described as intermittent (up to 50% of day) of moderate intensity (requires significant or complete modification to ADL); increased by changing positions; decreased by Prescribed Pain Medication.

FOURTH COMPLAINT
Left shoulder, Right shoulder

- Aching pain: 4/10
  Described as frequent (up to 75% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by lifting, bending and changing positions; decreased by Prescribed Pain Medication.

drop pelvis, activator on rest of spine

OBJECTIVE

PALPATORY FINDINGS

Spinal asymmetry/fixation: C1; C3; T5; T8; T12; Pelvis; SI; **T1. No findings: L1**.

Tenderness

Left Side	Right Side

- Shoulder (Mild-Moderate)
- Shoulder (Mild-Moderate)

Spasm/Hypertonicity

Left Side

- Cervical Spine (Minimal)
- Shoulder (Mild)

Right Side

- Shoulder
- Thoracic Spine (Minimal)
- Lower Back (Minimal)

ASSESSMENT

Stage of Care: Rehabilitative

DIAGNOSES

1. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)
2. Acute post-traumatic headache, intractable (G44.311)
3. Disorder of ligament, vertebrae (M24.28)
4. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
5. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
6. Strain of muscle, fascia and tendon at neck level, initial encounter (S16.1XXA)
7. Strain of muscle, fascia and tendon of lower back, initial encounter (S39.012A)
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13. Segmental and somatic dysfunction of lumbar region (M99.03)

PLAN

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of activator technique, Thompson technique was provided specifically to C1; C3; **T1**; **T5**; T8; T12; Pelvis; SI, to reduce pain reported at initial encounter by 25%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 25%, reduce muscle spasm, reduce chronic myofascial pain.
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- Ice/Heat (97010) consisting of hot pack was provided specifically to Neck, Left Scapula, Left Rib Cage, Right Scapula, Right Rib Cage, Back, to reduce pain reported at initial encounter by 25%, reduce muscle spasm, reduce swelling/inflammation.

Patient received care without incident.

(Electronic Signature)
Provider Name: Marita Thompson, DC
Date: 04/03/2024 12:35 PM

Iceberg Health Lawrenceville
1670 McKendree Church Road Suite 400B
Lawrenceville, GA 30043-4100
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Francisca Diaz Alvarado
Date of Birth: 10/3/1969

Provider: Marita Thompson, DC
Date of Service: 4/5/2024

SUBJECTIVE

PRIMARY COMPLAINT
Right hip, Left hip

- Aching pain: 6/10
  Described as intermittent to frequent (up to between 50% and 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by lifting, walking and changing positions; decreased by Prescribed Pain Medication.

SECOND COMPLAINT
Neck

- Pain: 5/10
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by changing positions; decreased by Prescribed Pain Medication.

THIRD COMPLAINT
Head

- Aching pain: 4/10
  Described as intermittent (up to 50% of day) of moderate intensity (requires significant or complete modification to ADL); increased by changing positions; decreased by Prescribed Pain Medication.

FOURTH COMPLAINT
Left shoulder, Right shoulder

- Aching pain: 4/10
  Described as frequent (up to 75% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by lifting, bending and changing positions; decreased by Prescribed Pain Medication.

pt has had 2 days off work and her arms and legs have been feeling a lot better so far

OBJECTIVE

PALPATORY FINDINGS

Spinal asymmetry/fixation: C1; T5; T8; T12; Pelvis; SI; **C4. No findings: C3; T1**.

Tenderness

Left Side	Right Side

- Shoulder (Mild-Moderate)
- Shoulder (Mild-Moderate)

Spasm/Hypertonicity

Left Side

- Cervical Spine (Minimal)
- Shoulder (Mild)

Right Side

- Shoulder
- Thoracic Spine (Minimal)
- Lower Back (Minimal)

ASSESSMENT

Stage of Care: Rehabilitative

DIAGNOSES

1. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)
2. Acute post-traumatic headache, intractable (G44.311)
3. Disorder of ligament, vertebrae (M24.28)
4. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
5. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
6. Strain of muscle, fascia and tendon at neck level, initial encounter (S16.1XXA)
7. Strain of muscle, fascia and tendon of lower back, initial encounter (S39.012A)
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9. Low back pain, unspecified (M54.50)
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11. Segmental and somatic dysfunction of cervical region (M99.01)
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PLAN

Based on subjective and objective data, today's treatment consists of:

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- Ice/Heat (97010) consisting of hot pack was provided specifically to Neck, Left Scapula, Left Rib Cage, Right Scapula, Right Rib Cage, Back, to reduce pain reported at initial encounter by 25%, reduce muscle spasm, reduce swelling/inflammation.

Patient received care without incident.

(Electronic Signature)
Provider Name: Marita Thompson, DC
Date: 04/05/2024 03:24 PM

Iceberg Health Lawrenceville
1670 McKendree Church Road Suite 400B
Lawrenceville, GA 30043-4100
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Francisca Diaz Alvarado
Date of Birth: 10/3/1969

Provider: Marita Thompson, DC
Date of Service: 4/10/2024

SUBJECTIVE

PRIMARY COMPLAINT
Right hip, Left hip

- Aching pain: 6/10
  Described as intermittent to frequent (up to between 50% and 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by lifting, walking and changing positions; decreased by Prescribed Pain Medication.

SECOND COMPLAINT
Neck

- Pain: 5/10
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by changing positions; decreased by Prescribed Pain Medication.

THIRD COMPLAINT
Head

- Aching pain: 4/10
  Described as intermittent (up to 50% of day) of moderate intensity (requires significant or complete modification to ADL); increased by changing positions; decreased by Prescribed Pain Medication.

FOURTH COMPLAINT
Left shoulder, Right shoulder

- Aching pain: 4/10
  Described as frequent (up to 75% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by lifting, bending and changing positions; decreased by Prescribed Pain Medication.

OBJECTIVE

PALPATORY FINDINGS

Spinal asymmetry/fixation: C1; C4; T8; Pelvis; SI; **T2; L3; L4; L5. No findings: T5; T12**.

Tenderness

Left Side	Right Side

- Shoulder (Mild-Moderate)
- Shoulder (Mild-Moderate)

Spasm/Hypertonicity

Left Side

- Cervical Spine (Minimal)
- Shoulder (Mild)

Right Side

- Shoulder
- Thoracic Spine (Minimal)
- Lower Back (Minimal)

ASSESSMENT

Stage of Care: Rehabilitative

DIAGNOSES

1. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)
2. Acute post-traumatic headache, intractable (G44.311)
3. Disorder of ligament, vertebrae (M24.28)
4. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
5. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
6. Strain of muscle, fascia and tendon at neck level, initial encounter (S16.1XXA)
7. Strain of muscle, fascia and tendon of lower back, initial encounter (S39.012A)
8. Cervicalgia (M54.2)
9. Low back pain, unspecified (M54.50)
10. Other muscle spasm (M62.838)
11. Segmental and somatic dysfunction of cervical region (M99.01)
12. Segmental and somatic dysfunction of thoracic region (M99.02)
13. Segmental and somatic dysfunction of lumbar region (M99.03)

PLAN

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of activator technique, Thompson technique was provided specifically to C1; C4; **T2**; T8; **L3**; **L4**; **L5**; Pelvis; SI, to reduce pain reported at initial encounter by 25%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 25%, reduce muscle spasm, reduce chronic myofascial pain.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to Back, Neck, to reduce pain reported at initial encounter by 25%, decrease significant facet pain, break-up adhesions, reduce chronic myofascial pain.
- Ice/Heat (97010) consisting of hot pack was provided specifically to Neck, Left Scapula, Left Rib Cage, Right Scapula, Right Rib Cage, Back, to reduce pain reported at initial encounter by 25%, reduce muscle spasm, reduce swelling/inflammation.

Patient received care without incident.

(Electronic Signature)
Provider Name: Marita Thompson, DC
Date: 04/10/2024 03:46 PM

Iceberg Health Lawrenceville
1670 McKendree Church Road Suite 400B
Lawrenceville, GA 30043-4100
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Francisca Diaz Alvarado
Date of Birth: 10/3/1969

Provider: Marita Thompson, DC
Date of Service: 4/12/2024

SUBJECTIVE

PRIMARY COMPLAINT
Right hip, Left hip

- Aching pain: 6/10
  Described as intermittent to frequent (up to between 50% and 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by lifting, walking and changing positions; decreased by Prescribed Pain Medication.

SECOND COMPLAINT
Neck

- Pain: 5/10
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by changing positions; decreased by Prescribed Pain Medication.

THIRD COMPLAINT
Head

- Aching pain: 4/10
  Described as intermittent (up to 50% of day) of moderate intensity (requires significant or complete modification to ADL); increased by changing positions; decreased by Prescribed Pain Medication.

FOURTH COMPLAINT
Left shoulder, Right shoulder

- Aching pain: 4/10
  Described as frequent (up to 75% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by lifting, bending and changing positions; decreased by Prescribed Pain Medication.

OBJECTIVE

PALPATORY FINDINGS

Spinal asymmetry/fixation: C1; C4; T8; L5; Pelvis; SI; **T4. No findings: T2; L3; L4**.

Tenderness

Left Side	Right Side

- Shoulder (Mild-Moderate)
- Shoulder (Mild-Moderate)

Spasm/Hypertonicity

Left Side

- Cervical Spine (Minimal)
- Shoulder (Mild)

Right Side

- Shoulder
- Thoracic Spine (Minimal)
- Lower Back (Minimal)

ASSESSMENT

DIAGNOSES

1. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)
2. Acute post-traumatic headache, intractable (G44.311)
3. Disorder of ligament, vertebrae (M24.28)
4. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
5. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
6. Strain of muscle, fascia and tendon at neck level, initial encounter (S16.1XXA)
7. Strain of muscle, fascia and tendon of lower back, initial encounter (S39.012A)
8. Cervicalgia (M54.2)
9. Low back pain, unspecified (M54.50)
10. Other muscle spasm (M62.838)
11. Segmental and somatic dysfunction of cervical region (M99.01)
12. Segmental and somatic dysfunction of thoracic region (M99.02)
13. Segmental and somatic dysfunction of lumbar region (M99.03)

PLAN

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of activator technique, Thompson technique was provided specifically to C1; C4; **T4**; T8; L5; Pelvis; SI, to reduce pain reported at initial encounter by 25%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 25%, reduce muscle spasm, reduce chronic myofascial pain.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to Back, Neck, to reduce pain reported at initial encounter by 25%, decrease significant facet pain, break-up adhesions, reduce chronic myofascial pain.
- Ice/Heat (97010) consisting of hot pack was provided specifically to Neck, Left Scapula, Left Rib Cage, Right Scapula, Right Rib Cage, Back, to reduce pain reported at initial encounter by 25%, reduce muscle spasm, reduce swelling/inflammation.
- **Manual Therapy (Ea 15 Min) (97140) consisting of soft tissue mobilization, trigger point therapy was provided specifically to Neck, Right Shoulder for 10 minutes (1 unit), to reduce pain reported at initial encounter by 25%, improve muscle strength from initial exam by 25%, break-up adhesions, reduce chronic myofascial pain.**

Patient received care without incident.

(Electronic Signature)
Provider Name: Marita Thompson, DC
Date: 04/12/2024 03:32 PM

Iceberg Health Lawrenceville
1670 McKendree Church Road Suite 400B
Lawrenceville, GA 30043-4100
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Francisca Diaz Alvarado
Date of Birth: 10/3/1969

Provider: Marita Thompson, DC
Date of Service: 4/15/2024

SUBJECTIVE

PRIMARY COMPLAINT
Right hip, Left hip

- Aching pain: 6/10
  Described as intermittent to frequent (up to between 50% and 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by lifting, walking and changing positions; decreased by Prescribed Pain Medication.

SECOND COMPLAINT
Neck

- Pain: 5/10
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by changing positions; decreased by Prescribed Pain Medication.

THIRD COMPLAINT
Head

- Aching pain: 4/10
  Described as intermittent (up to 50% of day) of moderate intensity (requires significant or complete modification to ADL); increased by changing positions; decreased by Prescribed Pain Medication.

FOURTH COMPLAINT
Left shoulder, Right shoulder

- Aching pain: 4/10
  Described as frequent (up to 75% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by lifting, bending and changing positions; decreased by Prescribed Pain Medication.

OBJECTIVE

PALPATORY FINDINGS

Spinal asymmetry/fixation: C1; C4; T4; L5; Pelvis; SI; **T7; L4. No findings: T8**.

Tenderness

Left Side	Right Side

- Shoulder (Mild-Moderate)
- Shoulder (Mild-Moderate)

Spasm/Hypertonicity

Left Side

- Cervical Spine (Minimal)
- Shoulder (Mild)

Right Side

- Shoulder
- Thoracic Spine (Minimal)
- Lower Back (Minimal)

ASSESSMENT

DIAGNOSES

1. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)
2. Acute post-traumatic headache, intractable (G44.311)
3. Disorder of ligament, vertebrae (M24.28)
4. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
5. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
6. Strain of muscle, fascia and tendon at neck level, initial encounter (S16.1XXA)
7. Strain of muscle, fascia and tendon of lower back, initial encounter (S39.012A)
8. Cervicalgia (M54.2)
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12. Segmental and somatic dysfunction of thoracic region (M99.02)
13. Segmental and somatic dysfunction of lumbar region (M99.03)

PLAN

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of activator technique, Thompson technique was provided specifically to C1; C4; T4; **T7**; **L4**; L5; Pelvis; SI, to reduce pain reported at initial encounter by 25%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 25%, reduce muscle spasm, reduce chronic myofascial pain.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to Back, Neck, to reduce pain reported at initial encounter by 25%, decrease significant facet pain, break-up adhesions, reduce chronic myofascial pain.
- Ice/Heat (97010) consisting of hot pack was provided specifically to Neck, Left Scapula, Left Rib Cage, Right Scapula, Right Rib Cage, Back, to reduce pain reported at initial encounter by 25%, reduce muscle spasm, reduce swelling/inflammation.
- Manual Therapy (Ea 15 Min) (97140) consisting of soft tissue mobilization, trigger point therapy was provided specifically to Neck, Right Shoulder for 10 minutes (1 unit), to reduce pain reported at initial encounter by 25%, improve muscle strength from initial exam by 25%, break-up adhesions, reduce chronic myofascial pain.

Patient received care without incident.

(Electronic Signature)
Provider Name: Marita Thompson, DC
Date: 04/15/2024 04:00 PM

Iceberg Health Lawrenceville
1670 McKendree Church Road Suite 400B
Lawrenceville, GA 30043-4100
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Francisca Diaz Alvarado
Date of Birth: 10/3/1969

Provider: Rita Patel, DC
Date of Service: 4/17/2024

SUBJECTIVE

PRIMARY COMPLAINT
Right hip, Left hip

- Aching pain: 6/10
  Described as intermittent to frequent (up to between 50% and 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by lifting, walking and changing positions; decreased by Prescribed Pain Medication.

SECOND COMPLAINT
Neck

- Pain: 5/10
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by changing positions; decreased by Prescribed Pain Medication.

THIRD COMPLAINT
Head

- Aching pain: 4/10
  Described as intermittent (up to 50% of day) of moderate intensity (requires significant or complete modification to ADL); increased by changing positions; decreased by Prescribed Pain Medication.

FOURTH COMPLAINT
Left shoulder, Right shoulder

- Aching pain: 4/10
  Described as frequent (up to 75% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by lifting, bending and changing positions; decreased by Prescribed Pain Medication.

OBJECTIVE

PALPATORY FINDINGS
Spinal asymmetry/fixation: C1; C4; T4; T7; L4; L5; Pelvis; SI.
Tenderness

Left Side | Right Side

- Shoulder (Mild-Moderate)
- Shoulder (Mild-Moderate)

Spasm/Hypertonicity

Left Side

- Cervical Spine (Minimal)
- Shoulder (Mild)

Right Side

- Shoulder
- Thoracic Spine (Minimal)
- Lower Back (Minimal)

ASSESSMENT

DIAGNOSES

1. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)
2. Acute post-traumatic headache, intractable (G44.311)
3. Disorder of ligament, vertebrae (M24.28)
4. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
5. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
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7. Strain of muscle, fascia and tendon of lower back, initial encounter (S39.012A)
8. Cervicalgia (M54.2)
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10. Other muscle spasm (M62.838)
11. Segmental and somatic dysfunction of cervical region (M99.01)
12. Segmental and somatic dysfunction of thoracic region (M99.02)
13. Segmental and somatic dysfunction of lumbar region (M99.03)

PLAN

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of activator technique, Thompson technique was provided specifically to C1; C4; T4; T7; L4; L5; Pelvis; SI, to reduce pain reported at initial encounter by 25%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 25%, reduce muscle spasm, reduce chronic myofascial pain.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to Back, Neck, to reduce pain reported at initial encounter by 25%, decrease significant facet pain, break-up adhesions, reduce chronic myofascial pain.
- Ice/Heat (97010) consisting of hot pack was provided specifically to Neck, Left Scapula, Left Rib Cage, Right Scapula, Right Rib Cage, Back, to reduce pain reported at initial encounter by 25%, reduce muscle spasm, reduce swelling/inflammation.
- Manual Therapy (Ea 15 Min) (97140) consisting of soft tissue mobilization, trigger point therapy was provided specifically to Neck, Right Shoulder for **15** minutes (1 unit), to reduce pain reported at initial encounter by 25%, improve muscle strength from initial exam by 25%, break-up adhesions, reduce chronic myofascial pain.

Patient received care without incident.

(Electronic Signature)
Provider Name: Rita Patel, DC
Date: 04/17/2024 10:46 AM

Iceberg Health Lawrenceville
1670 McKendree Church Road Suite 400B
Lawrenceville, GA 30043-4100
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Exam

Patient Name: Francisca Diaz Alvarado
Date of Birth: 10/3/1969

Provider: Rita Patel, DC
Date of Service: 4/24/2024

SUBJECTIVE

PRIMARY COMPLAINT
Right hip, Left hip

- Aching pain: **From 6 to 5/10 (10% decrease)**
  Described as intermittent to frequent (up to between 50% and 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by lifting, walking and changing positions; decreased by Prescribed Pain Medication.

SECOND COMPLAINT
Right shoulder, **Right scapula**, Neck

- Pain: 5/10
  Described as **intermittent to frequent (up to between 50% and 75% of day)** of moderate intensity (requires significant or complete modification to ADL); increased by changing positions; decreased by Prescribed Pain Medication.
- **Stiffness: 4/10**
  Described as intermittent to frequent (up to between 50% and 75% of day) of moderate intensity (requires significant or complete modification to ADL).

THIRD COMPLAINT
Head

- Aching pain: **From 4 to 2/10 (20% decrease)**
  Described as intermittent (up to 50% of day) of moderate intensity (requires significant or complete modification to ADL); increased by changing positions; decreased by Prescribed Pain Medication.

FOURTH COMPLAINT
Right shoulder

- Aching pain: **From 4 to 5/10 (10% increase)**
  Described as frequent (up to 75% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by lifting, bending and changing positions; decreased by Prescribed Pain Medication.

The patient is being discharged today after a course of care with residual deficits in function from the injuries sustained from their Motor Vehicle Accident.
According to the American Medical Association (AMA) Guides to the Evaluation of Permanent Impairment 5th edition this patient may have a Permanent whole-body impairment of up to 25%.
In order to prevent further damage this patient will have to receive regular treatment at the rate of 2 times per month for life.
Approximate $48,780 in conservative lifetime treatment costs.
Lifetime Treatment Costs = [Avg. Life expectancy - Current Age] X [2($75.00) X 12]

The average Male Life Expectancy is 75.3 years.
The average Female Life Expectancy is 81.1 years.
According to the above biomechanical assessment, there are findings of alteration of motion segment integrity (AOMS)I) at the following levels: C3-C4, C4-C5 and C6-C7. Consequently, this patient may be ratable for a permanent injury upon reaching maximal medical improvement.

OBJECTIVE

PALPATORY FINDINGS

Spinal asymmetry/fixation: C1; C4; T4; T7; L4; Pelvis; **C6. No findings: L5; SI**.

Tenderness

Left Side
- Shoulder (Mild-Moderate)

Right Side
- Shoulder (Mild-Moderate)

Spasm/Hypertonicity

Left Side
- Shoulder **(decreased 10-20%)**

Right Side
- **Cervical Spine (mild-moderate)**
- Shoulder **(increased 10-20% to mild)**
- Thoracic Spine (Minimal)
- Lower Back (Minimal)

Edema

Left Side

Right Side
- **Cervical Spine (moderate)**
- **Shoulder (mild-moderate)**

ORTHOPEDIC AND NEUROLOGICAL TESTS

Test	Left	Right	Comment
Cervical distraction [1]	WNL	Positive	
Foraminal compression test[2]	Positive	Positive	
Shoulder depression test[3]	WNL	Positive	

[1]Relief of radicular pain suggests nerve root compression. Decreased local pain could indicate facet impingement. Increased local pain could indicate strain or sprain.

[2]Radicular pain could indicate pressure on the nerve root. Localized pain could indicate sprain or strain or foraminal encroachment.

[3]Pain in the arm could indicate adhesions of the nerve roots. Local pain could indicate sprain or strain or adhesions of the shoulder joint capsule.

Nachlas test and Patrick test are within normal limits.

RANGE OF MOTION

Cervical ROM - estimated

Test	Measurement	Limited By	Comment
Flexion	40° (80% of normal)		
Left lateral flexion	**40° (88% of normal, increased by 14%)**	Pain	
Extension	**50° (83% of normal, increased by 11%)**	Pain	

Right lateral flexion	35° (77% of normal)	Pain	
Left rotation	70° (87% of normal)		
Right rotation	70° (87% of normal)		

Lumbar ROM - estimated

Test	Measurement	Limited By	Comment
Flexion	**55° (91% of normal, increased by 22%)**		
Extension	**20° (80% of normal, increased by 33%)**	Pain	
Left lateral flexion	**20° (80% of normal, increased by 33%)**		
Right ateral flexion	**20° (80% of normal, increased by 33%)**	Pain	
Left rotation	25° (83% of normal)		
Right rotation	25° (83% of normal)		

DERMATOMES

Anterior

Location	Test	Result
Left-Lumbar	pinwheel-sensitivity	Paraesthesia
Right-Lumbar	pinwheel-sensitivity	Paraesthesia

ASSESSMENT

DIAGNOSES

1. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)
2. Acute post-traumatic headache, intractable (G44.311)
3. Disorder of ligament, vertebrae (M24.28)
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7. Strain of muscle, fascia and tendon of lower back, initial encounter (S39.012A)
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10. Other muscle spasm (M62.838)
11. Segmental and somatic dysfunction of cervical region (M99.01)
12. Segmental and somatic dysfunction of thoracic region (M99.02)
13. Segmental and somatic dysfunction of lumbar region (M99.03)

LONG-TERM GOALS

- Be able to lift without pain: **Progressed from 0% to 80% of goal achieved.**

Patient condition is **slightly regressed as per subjective and objective findings** since the prior visit, She is **progressing slowly**. Intensity is **slightly-decreased** since the prior visit.

PLAN

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of activator technique, Thompson technique was provided specifically to C1; C4; T4; T7; L4; Pelvis, to reduce pain reported at initial encounter by 25%.

- Electrical Stimulation (97014) consisting of interferential was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 25%, reduce muscle spasm, reduce chronic myofascial pain.
- Ice/Heat (97010) consisting of hot pack was provided specifically to Neck, Left Scapula, Left Rib Cage, Right Scapula, Right Rib Cage, Back, to reduce pain reported at initial encounter by 25%, reduce muscle spasm, reduce swelling/inflammation.
- Manual Therapy (Ea 15 Min) (97140) consisting of soft tissue mobilization, trigger point therapy was provided specifically to Neck, Right Shoulder for 15 minutes (1 unit), to reduce pain reported at initial encounter by 25%, improve muscle strength from initial exam by 25%, break-up adhesions, reduce chronic myofascial pain.
- **Established Patient evaluation and management service, 99213, was provided today.**

Patient received care without incident.

(Electronic Signature)
Provider Name: Rita Patel, DC
Date: 04/24/2024 04:29 PM